

eCBHFA

Violence prevention and response module

Three-day training of facilitators

Learning objectives

At the completion of this topic volunteers will be able to:

- Deliver a two-day training to eCBHFA Volunteers on the Violence Prevention and Response module.
- Explain the various forms of interpersonal violence against girls, boys, women and men: physical, psychological, sexual-gender-based violence, and neglect.
- Identify key factors of self-directed violence.
- Define populations, including by age and gender, who are at highest risk of violence.
- Recognize that violence is predictable and preventable.
- Identify practical, local actions that individuals and communities can take to prevent violence.
- Describe options for handling and referring disclosures of violence.

This module provides a basic introduction to different types of violence. Additional local resources should be explored to go further into each type of violence.

Main learning points

Key learning points

1. Everyone deserves to be safe from all violence: physical, sexual, psychological, and neglect.
2. Girls, boys, women and men need to be treated equally and with respect.
3. Hitting another person is harmful and abuses their human rights.
4. Sexual and gender-based violence should never be tolerated, it is a violation of our basic human rights: dignity and safety.
5. Psychological violence is damaging to a person's self-esteem, it often has long-lasting consequences.
6. Harming yourself hurts you and those you care about.
7. Violence can be prevented – help and laws exist.

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Complementary eCBHFA Modules

eCBHFA modules that are closely associated with and complement the Violence Prevention and Response module:

- 1) Female Genital Cutting
- 2) Psychosocial Support
- 3) Mother, Child and Newborn Health

Based on local needs and context, these modules can be implemented along with the Violence Prevention and Response module in order to enhance impact.

Suggested outline of activities

Day 1: Problem identification (7.5 hours)		
1. Introduction	Present information from the topic summary	1 hour (60 min)
2. Discussion	Discuss the types of violence and vulnerable populations.	4 hours, 15 min (255 min)
	Part 1: Provide overview of Violence Prevention and Response.	1 hour, 15 min (75 min)
	Part 2: Define specific types of violence.	45 min
	Part 3: Explore in detail each type of violence.	2 hours, 15 min (135 min)
3. Closing	Closing of day and feedback.	15 min
Day 2: Problem implementation (7.5 hours)		
4. Check your understanding	Self-assessment from day 1.	15 min
5. Lesson learned	Review global and regional lessons learned using the module.	1 hour (60 min)
6. Ways of learning	Discuss different ways of learning.	30 min
7. Activity in the Red Cross Red Crescent	Practice delivering the lesson cards and identify appropriate micro-projects.	3 hours, 30 min (210 min)
8. Closing	Closing of the day and feedback.	15 min

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Day 3: Planning action (8 hours)		
9. Check your understanding	Review of day 2.	15 min
10. Protection laws	Review local protection laws and international instruments.	30 min
11. Informal support systems	Identify informal community support systems.	30 min
12. Handling disclosures	Define consideration and scenarios for handling disclosures, and how to ensure self-care.	2 hours (120 min)
13. Expert resources	Organize an expert resource panel to discuss local partnerships and support.	1 hour (60 min)
14. Planning for local action	Develop localized plans of action to implement the module.	1 hour, 30 min (90 min)
15. Closing	Define personal commitments, complete satisfaction surveys, and thank participants.	30 min

Facilitator tips:

- Violence, especially sexual and gender based violence (SGBV), can be a sensitive topic; it is essential to know that some participants may have experienced violence in their own lives. Therefore, some participants may at times feel painful emotions, such as fear, shame and mistrust. The topic of violence should be handled with care and compassion. At the beginning of the training, each participant should agree to confidentiality, understanding that information discussed during the session shall not be shared outside the group.
- If someone discloses a violent experience, listen carefully to the person and refer them to local support systems, as appropriate. Ensure that you apply the survivor centred approach: treat the person in a dignified way, ensure this person is able to disclose in a safe space, respect the person's decision and reflect a non-judgemental attitude towards the person's experience.
- Ensure you have a copy of local referral services that participants can access if they want specific psychosocial, health or other protection support. This information should be regularly updated before trainings.

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- Ensure the learning environment is safe for participants by keeping an eye on participants' reactions to different topics, grouping participants appropriately to reduce frictions, re-directing inappropriate or unsafe conversations, and giving breaks or using energizers as needed.
- Recognize that the perspectives, realities and needs of women and men in the training may be different; having participation based on these different perspectives will strengthen the training. It is important to discuss what will make a gender-specific safe environment especially for topics like SGBV.
- Remember to focus the discussions so that solutions remain practical, culturally-appropriate, and achievable in the places where participants live and work.
- If a translator is required to support the training, increase the training time by 40%.

Materials (put on memory stick or print out)

- Topic Summary.
- Violence Prevention and Response lesson cards (7 cards).
- Agenda.
- Two-day training curriculum and power point.
- Powerpoint tool.
- Flipchart paper, coloured markers and post-it notes.
- Copies of the local SGBV, child protection and self-harm laws for the territory or country.
- Local referral pathways for SGBV and violence against children.
- Convention on the Rights of the Child and the Convention on the Elimination of Discrimination against Women.
- IFRC Minimum Standards for Protection, Gender and Inclusion in Emergencies
- IFRC or National Society Prevention of Sexual Exploitation and Abuse (PSEA) and Child Protection policies.
- IFRC International Disaster Response Law and SGBV reports (global or specific to region).
- IFRC International Conference Resolution 3: Joint Action on SGBV Prevention and Response during Emergencies (2015).
- Self Assessment Tool: either as a hand-out or power-point.
- Satisfaction questionnaire.

Key definitions

Violence

Violence is the use of force or power, either as an action or omission in any setting, threatened, perceived or actual against oneself, another person, a group, or a community that either results in or has a high likelihood of resulting in death, physical injury, psychological or emotional harm, mal-development or deprivation.

In essence, violence between people (interpersonal violence) is when one person uses his or her power, in any setting, to cause harm physically, sexually or psychologically to a person or group of people.

Gender

Refers to the social differences between females and males throughout their life cycles. Although deeply rooted in every culture, these social differences between females and males are changeable over time and are different both within and between cultures. Gender determines the roles, power and resources for females and males in any culture. In most societies, there are differences and inequalities between women and men in responsibilities assigned, activities undertaken, access to and control over resources, as well as decision-making opportunities

Gender equality

Refers to the equal rights, responsibilities and opportunities of women and men and girls and boys. Equality does not mean that women and men will become the same but that women's and men's rights, responsibilities and opportunities will not depend on whether they are born male or female.

Gender identity

Refers to each person's deeply felt internal and individual experience of gender, which may or may not correspond with the sex assigned at birth. It includes the personal sense of the body (which may involve, if freely chosen, modification of bodily appearance or function by medical, surgical or other means) and other expressions of gender, including dress, speech and mannerisms.

Sexual minorities

An umbrella term used to describe persons subject to discrimination and abuse due to their non-conformance with prevailing gender norms. Sometimes used in place LGBTI, which is an acronym for lesbian, gay, bisexual and transgender, and intersex persons.

Lesbian

A woman whose enduring physical, romantic and/or emotional attraction is to other women. Some women prefer to be referred to as 'gay' or 'gay women'.

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Gay

Used to describe people whose enduring physical, romantic and/or emotional attractions are to people of the same sex (e.g. gay man, gay people). Often used to describe a man who is sexually attracted to other men, but may be used to describe lesbians as well.

Bisexual

An individual who is physically, romantically and/or emotionally attracted to both men and women. Bisexuals need not have had equal sexual experience with both men and women. In fact, they need not have had any sexual experience at all to identify as bisexual.

Transgender

An umbrella term for people whose gender identity and/or gender expression differs from the sex they were assigned at birth. The term may include but is not limited to: transsexuals, crossdressers and other gender-variant people. Transgender people may identify as female-to-male (FTM), male-to-female (MTF) or other genders altogether. Transgender people may or may not decide to alter their bodies hormonally and/or surgically.

Intersex

Refers to a condition of having sexual anatomy that is not considered 'standard' for a male or female. 'Intersex' can be used as an umbrella term covering differences of sexual development, which can consist of diagnosable congenital conditions in which development of chromosomal, gonadal or anatomic sex is atypical. The term 'intersex' is not interchangeable or a synonym for transgender.

Sexual and Gender-based violence (SGBV)

Sexual and gender-based violence (SGBV) is an umbrella term for any harmful act that is perpetrated against a person's will and that is based on socially ascribed (i.e. gender) differences between males and females. It includes acts that inflict physical, sexual or mental harm or suffering, threats of such acts, coercion, and other deprivations of liberty. These acts can occur in public or in private. The term is primarily used to underscore the fact that structural, gender-based power differentials between males and females around the world place females at risk for multiple forms of violence. Despite their prevalence, incidents of GBV are often invisible, as taboos, stigma, feelings of guilt or shame, fear of retribution, and the unavailability of support or lack of information about available support often prevent survivors from coming forward.

While women and girls are disproportionately affected, men and boys can also be victims/survivors of SGBV and other factors such as age, disability, displacement, religion, ethnicity, sexual orientation and gender identity, among others, may increase the risk. Nevertheless, most practitioners

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agree that while humanitarian actors must analyse different gendered vulnerabilities that may put men, women, boys and girls at heightened risk of violence and ensure care and support for all survivors, special attention should be given to females due to their documented greater vulnerabilities to SGBV, the overarching discrimination they experience, and their lack of safe and equitable access to humanitarian assistance. The term SGBV is also used by some actors to describe violence perpetrated against lesbian, gay, bisexual, transgender and intersex (LGBTI) persons that is driven by a desire to punish those seen as defying gender norms. The acronym 'LGBTI' encompasses a wide range of identities that share an experience of falling outside societal norms due to their sexual orientation and/or gender identity. Lesbians and transgender women are at particular risk because of gender inequality and power relations within families and wider society. Homophobia and transphobia not only contribute to this violence but also significantly undermine LGBTI survivors' ability to access support.

Sexual violence

Includes, at least, rape/attempted rape, sexual abuse and sexual exploitation. Sexual violence is "any sexual act, attempt to obtain a sexual act, unwanted sexual comments or advances, or acts to traffic a person's sexuality, using coercion, threats of harm or physical force, by any person regardless of relationship to the victim, in any setting, including but not limited to home and work." Sexual violence takes many forms, including rape, sexual slavery and/or trafficking, forced pregnancy, sexual harassment, sexual exploitation and/or abuse, and forced abortion.

Rape

Physically forced or otherwise coerced penetration—even if slight—of the vagina, anus or mouth with a penis or other body part. It also includes penetration of the vagina or anus with an object. Rape includes marital rape and anal rape/sodomy. The attempt to do so is known as attempted rape. Rape of a person by two or more perpetrators is known as gang rape.

Sexual exploitation and abuse

'Sexual exploitation' means any actual or attempted abuse of a position of vulnerability, differential power or trust for sexual purposes, including, but not limited to, profiting monetarily, socially or politically from the sexual exploitation of another. Some types of forced and/or coerced prostitution can fall under this category. The term 'sexual abuse' means the actual or threatened physical intrusion of a sexual nature, whether by force or under unequal or coercive conditions. In the humanitarian world, 'sexual exploitation and abuse' (SEA) commonly refer to acts of sexual exploitation and sexual abuse committed by United Nations, NGO, and inter-governments (IGO) personnel against the affected population.

Sexual harassment

Unwelcome sexual advances, requests for sexual favours, and other verbal or physical conduct of a sexual nature

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Sexual assault

Any form of non-consensual sexual contact that does not result in or include penetration. Examples include: attempted rape, as well as unwanted kissing, fondling, or touching of genitalia and buttock

Trafficking in persons

The recruitment, transportation, transfer, harbouring or receipt of persons, by means of the threat or use of force or other forms of coercion, of abduction, of fraud, of deception, of the abuse of power or of a position of vulnerability or of the giving or receiving of payments or benefits to achieve the consent of a person having control over another person, for the purpose of exploitation. Exploitation shall include, at a minimum, the exploitation of the prostitution of others or other forms of sexual exploitation, forced labour or services, slavery or practices similar to slavery, servitude or the removal of organs

Domestic violence and intimate partner violence

While these terms are sometimes used interchangeably, there are important distinctions between them. 'Domestic violence' is a term used to describe violence that takes place within the home or family between intimate partners as well as between other family members. 'Intimate partner violence' applies specifically to violence occurring between intimate partners (married, cohabiting, boyfriend/girlfriend or other close relationships), and is defined by WHO as behaviour by an intimate partner or ex-partner that causes physical, sexual or psychological harm, including physical aggression, sexual coercion, psychological abuse and controlling behaviours.⁴ This type of violence may also include the denial of resources, opportunities or services.

Child, early and forced marriage

Forced marriage is the marriage of an individual against her or his will. Child/early marriage is a formal marriage or informal union before age 18. Even though some countries permit marriage before age 18, international human rights standards classify these as child marriages, reasoning that those under age 18 are unable to give informed consent. Therefore, child marriage is a form of forced marriage as children are not legally competent to agree to such unions.

Multi-sectoral approach to SGBV

Programming experiences from the field have revealed that no single sector or agency can adequately address all elements of SGBV response. A multi-sectoral model calls for holistic inter-organizational and inter-agency efforts that promote participation of people of concern, interdisciplinary and inter-organizational cooperation, and collaboration and coordination across key sectors, including (but not limited to) health, psychosocial, legal/justice and security. A key principle underlying the multi-sectoral approach is that the rights and needs of survivors are pre-eminent, in terms of access to respectful and supportive services, guarantees of confidentiality and safety and the ability to determine the course of action for addressing the incident.

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- The health sector should train providers across a wide variety of health services to recognize and address GBV; ensure same sex data collectors for individuals who have been exposed to violence; respond to the immediate health and psychological needs of survivors; institute protocols for treatment, referral and data collection and documentation that guarantee confidentiality; provide violence-related treatment free of cost; and be prepared to provide forensic evidence and testimony in court when authorized by the individual.
- The psychosocial sector should be able to provide ongoing psychological assistance, which requires the training and on-going supervision of psychologists, social workers and community services workers, and facilitate referrals for other services.
- The legal/justice sectors should be able to provide free or low-cost legal counselling, representation and other court support to survivors of violence; review and revise laws related to SGBV; enforce laws that protect survivors and punish perpetrators; monitor court cases and judicial processes; provide orders of protection and other legal safety mechanisms for survivors; and monitor perpetrators' compliance with court-ordered rehabilitation (e.g., batterer programs).
- Within the security sector police, military and other security personnel should be educated about SGBV and be trained on how to appropriately intervene when faced with cases. Police should have private rooms to ensure confidentiality and safety of survivors reporting a victimization; ensure same sex data collectors; institute protocols for referrals to other sectors; collect standardized and disaggregated data on incidents; and create specialized units to address SGBV.

Human Rights based approach

A human rights-based approach seeks to analyse the root causes of problems and to redress discriminatory practices that impede humanitarian intervention. This approach is often contrasted with the needs-based approach, in which interventions aim to address practical, short-term emergency needs through service delivery. By contrast, the human rights-based approach views affected populations as 'rights holders', and recognizes that these rights can be realized only by supporting the long-term empowerment of affected populations through sustainable solutions. This approach seeks to attend to rights as well as needs; how those needs are determined and addressed is informed by legal and moral obligations and accountability. Humanitarian actors, along with states (where they are functioning), are seen as 'duty-bearers' who are bound by their obligations to encourage, empower and assist 'rights-holders' in claiming their rights.

Victim or survivor?

Both terms 'survivor' and 'victim' refer to a person who has experienced gender-based violence. The terms can be used interchangeably, however, 'survivor' is the preferred term since it recognises the agency of the person who have experienced violence and focuses on individual capacity.

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Perpetrator

Person, group or institution that directly inflicts or otherwise supports violence or other abuse inflicted on another against his/her will.

SGBV referral pathway

A 'SGBV referral pathway' is a flexible mechanism that safely links GBV survivors to supportive and competent services, such as medical care, mental health and psychosocial support, police assistance and legal/justice support.

Active Listening

Active listening is a communication technique used in counselling, training, and conflict resolution. It requires that the listener fully concentrate, understand, respond and then remember what is being said.

Cultural sensitivity

Cultural sensitivity begins with a recognition that there are differences between cultures. These differences are reflected in the ways that different groups have different values, behaviours and experiences and communicate and relate to one another in different ways. To recognizing differences among cultures is thus important, but as service providers and researchers we also need to be aware of that differences also exist within cultures. A larger group may for example share common historic and geographic experiences, but individuals *within* the group may share nothing beyond that. Therefore, we need to be aware of these differences, respect them, and work within these parameters in interaction with our target groups. To do so, it is crucial to also recognise one's own cultural values, draw parallels where possible and identify any prejudices and stereotypes that may prevent us from communicating, reaching out to and understand the people of concern effectively

Self-care

People who work directly or indirectly with survivors of SGBV might experience pain and distress through listening to stories of violence, rapes, threats, and humiliation. To reduce the potential psychological strain experienced, being aware of your own reactions and warning signs and take appropriate actions in time eg. by seeking support, is key. This is what we call self-care. These are four key principles of self-care and tips for you as a field researcher on SGBV of how to take care of yourself in your work;

- *Awareness*: Learn to recognize signs of unhealthy stress and distress, e.g. nervous tension, anger, distractedness, feeling down, headaches, a more negative outlook on the world.
- *Balance*: Planning for sufficient time to rest and relax, and to cultivate private relationships.
- *Connection*: Building supportive relationships with co-workers, friends, family and others.
- *Discussing self-care*: Discuss experience among colleagues, and considering what collective measures can be taken to cope with distressful experiences and how one can reduce and mitigate the potential negative effects of the work.

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Child protection¹

Child protection refers to an activity or initiative for the purpose of preventing or responding to a specific incident of child abuse

Violence against children

Violence against children is one or more of the following actions against a child:

- physical abuse
- sexual abuse
- emotional abuse
- neglect
- exploitation
- bullying
- harassment

Psychological violence

Physical violence is when a person in a position of power, authority or trust repeatedly attacks a person's self-esteem verbally or non-verbally. This includes rejecting, degrading, isolating, ignoring, terrorizing, corrupting and/or exploiting a person. While physical scars may heal, emotional hurt may continue to cause pain long after the violence occurs. While emotional violence may leave no visible scars, it may be a predictor of self-harm in the person who is emotionally abused.

Neglect

Neglect is when a person's basic needs are not met and their health and development are affected. This applies most frequently to children and the elderly who rely on others for care and who are particularly at risk. Basic needs include safety/protection, nutrition, shelter, good hygiene, medical and dental care, adequate education, kindness and love,

Self-directed violence

Self-directed violence is when a person deliberately harms oneself. This can include taking one's own life, attempting to take one's own life, or self-mutilation.

Child

Child is defined by the *United Nations Convention on the Rights of the Child* as every human being under the age of 18 years unless, under national law, the age of majority is attained earlier. (Ensure you know the legal definition of a child that is used in the location of your training).

¹ IFRC Child Protection Policy

Topic summary

For use in the section on “Introduction.” Additional information can also be found in the lesson cards and the resources listed in the section on “Materials.”

IFRC’s Strategic Framework for Violence Prevention and Response

The IFRC has included Violence Prevention and Response, including SGBV prevention and response, PSEA, and child protection as part of its strategic frameworks. This includes: Strategy 2020 strategic aim three (promoting social inclusion and cultures of nonviolence and peace); IFRC Plan and Budget 2020; the Strategy on Violence Prevention, Mitigation and Response; Strategic Framework on Gender and Diversity; a Resolution on SGBV Prevention and Response, a Child Protection Action Plan; and through integration into many program frameworks including the Principles and Rules for Humanitarian Assistance. Many National Societies also include issues relating to violence prevention and response in their own strategic frameworks and programming.

Resolution on SGBV Prevention and Response

In 2015 the Red Cross and Red Crescent Movement developed a Resolution on Prevention and Response to SGBV in Emergencies. It outlines the areas of action that the IFRC, ICRC and National Societies can pursue, including in partnership with governments, to address the risk of SGBV and to better protect survivors.

Levels of violence in the world

Examples of global statistics on violence:

- Every day 4,200 people globally die from violence – approximately 1.6 million people a year. More than 90 per cent of these deaths occur in low and middle-income countries. When we think of the SEM, we see that approximately 2,300 people every day die from suicide – self-harm at the individual level, 1,500 people every day die from interpersonal violence in the family or between friends or groups they know and 400 people every day die from collective violenceⁱ at the community level and beyond. In looking at the SEM, we can see that the CBHFA volunteer can be incredibly helpful in addressing people at these different levels to better identify people suffering from violence.
- Each year, 16 million people suffer injury from violence that requires medical attention in a hospital.ⁱⁱ
- Between 500 million–1.5 billion children experience violence each year.ⁱⁱⁱ
- An estimated one in 20 elderly people experience abuse each year.^{iv}
- At least one out of every three women around the world has been beaten, coerced into sex, or otherwise abused in her lifetime—with the abuser commonly being someone known to her.^v
- Males are victims of nearly 80 per cent of all homicides, 60 per cent of suicides, and 80 per cent of violence-related injuries.^{vi}

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Do no harm

“Do no harm” is a key principle of humanitarian work and the work of CBHFA teams. Do no harm means that we must actively avoid or minimize any negative effects of an intervention, including activities like delivering this module if the community is not properly sensitised to address these very sensitive issues. We must take care to recognise our responsibility when we teach this content as it may upset some and put those we aim to serve at greater risk of increased danger or abuse of their rights. For this module that means, no actions are taken that can put girls, boys, women or men at higher risk of violence in their community or homes. To do this, we advise that communities are sensitised on the behaviour change module of eCBHFA and special attention is given to understanding cultural norms around different types of violence outlined above. If a culture or group has a history of practicing one of these types of abuse or violence and claim that it is a part of the cultural and societal norms, we strongly advise that you conduct a session with leaders and the community committee before teaching this content to any group. It is critical to identify the existing practices and explain the risks associated with each type of violence and how and why RCRC wishes to address this within CBHFA.

An example of this might be a culture where there is an expectation that teachers and parents and people of authority must hit children to get them to do as they are told. In talking to a community to define what type of hitting, frequency of hitting and the deliverers of the violence is acceptable in that culture – and why – often the frank discussion allows many in the group to recognise the hitting as the violence that it is. By talking about the norms and defining what the norm is and the rationale for it, it often can lose its powerful hold on a group. By then explaining the risks or problems associated with the practices, the norm can start to lose its hold on the group. This takes them past the knowledge stage of behaviour change to approach the approval stage. By exposing them then to people who suffered significant harm from the cultural norm or violence, the CBHFA volunteer can get them to advance to further stages of behaviour change. It is highly recommended and encouraged that technical experts in the area of child protection and SGBV are closely engaged and included in the implementation of this module – this may reduce the risk of causing further harm in discussing such sensitive topics.

Lesson cards and their messages

Lesson card 1: Everyone should be safe from all violence – physical, sexual and gender-based, psychological, and neglect.

Everyone, regardless of their age, gender, or any other status, deserves to be safe from all types of violence. Laws exist, in most countries, to keep people safe from violence. This is important because violence has many damaging consequences. A key part of violence is someone misusing their power to hurt others. Populations at high risk include: girls, boys, women, the elderly, people with disabilities, and anyone who is marginalized or seen as 'different.'

In disasters the risk of violence, such as SGBV and violence against children, increases.

Bystanders have an important role to play to prevent and respond to violence. Anyone suspecting violence against a child is required to make a report through appropriate local protection agencies.

It is essential to conduct detailed and appropriate assessments to identify and support local populations at risk.

Lesson card 2: Girls, boys, women and men need to be treated equally and with respect.

Everyone, of any gender, needs to be treated equitably and with respect. Power differences between genders can lead to SGBV. Therefore, understanding different gender norms, vulnerabilities and realities is essential for preventing and responding to violence.

Discrimination, including gender-based inequalities, increases the risk of violence, especially against girls and women. Men and boys have a valuable role in ensuring girls and women are treated equitably and with respect.

Valuing girls and women the same as boys and men means that individuals, families and communities will be safer and healthier. By providing respect through equitable access to basic essential services like education, health, psychosocial support, and the development of economic opportunities, the whole community benefits.

Lesson card 3: Hitting another person is harmful and abuses their human rights.

Physical violence can have a physical and psychological impact on the victim/survivor. For example, there are many options to discipline children that do not include physical harm regardless of cultural norms.

Adults can communicate and deal with conflict with other adults and with children without hitting or using violence.

When people witness violence within their family or community it has long lasting psychosocial impacts for those being hit, those witnessing the violence, and those causing the violence.

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Lesson card 4: Sexual and gender-based violence should never be tolerated, because it is a violation of our basic human rights: dignity and safety.

Research shows girls and women are disproportionately affected by SGBV, however, SGBV can happen to people of all ages, to any gender including men and boys, and/or people of any sexual orientation or background or status including people with disabilities.

No child (girl or boy) or adult (woman or man) of any background or status should be touched in unwanted, inappropriate or unsafe ways that violate a person's personal space or boundaries.

SGBV is an issue faced by all the communities. It is often life threatening and impacts a survivor's daily life, dignity, rights, livelihoods, psychosocial wellbeing, and health.

Offenders of SGBV are abusing their power in order to harm and traumatize another person.

SGBV is often kept secret because of social pressure, stigma, a lack of safe referral options and a belief that violence is the fault of the survivor. Although many offenders threaten to harm survivors if they tell anyone.

It must be clearly and repeatedly stated that SGBV is never the fault of the survivor.

When handling disclosures, it is important to ensure the survivor is in a safe space (away from harm), that the information shared is kept confidential, that the survivor's decision is respected and not discriminated against.

Lesson card 5: Psychological violence is damaging to a person's self-esteem and often has long-lasting consequences.

Psychological violence is part of all other forms of interpersonal violence: physical, SGBV, and neglect. Psychological violence is also its own type of violence when people are constantly rejected, insulted, degraded, or isolated.

There are many consequences of psychological violence including damaging a person's self-esteem; emotional scars can sometimes take longer to heal than physical ones.

Lesson card 6: Harming yourself hurts you and those you care about.

Self-harm can be prevented.

Self-harm through suicide is the leading cause of violent deaths in the world. Over 800,000 people die from suicide each year. The populations at greatest risk include the elderly and young people who feel they have few choices in their lives. Both males and females are at risk of suicide and other self-harm.

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Because of the stigma associated with suicide, suicide deaths are often not reported as suicides or the cause of death is reported by family as reasons that are less stigmatizing. In many places suicide is illegal and this further adds to the stigma and the need to keep the cause of death a secret. As a result, the true scope of suicide is hard to measure.

When a person harms their own body their families and those that care about them also suffer out of loss, worry and fear. Family and others that care about the person can also become stigmatized because of their relationship to someone who has hurt themselves.

Lesson card 7: Violence can be prevented – help and laws exist.

We all have a responsibility to prevent and respond to violence wherever we live, work and play. While it is a challenging problem due to cultural norms and expectations, violence is a preventable public health problem. In the same way as other public health problems like malaria, measles, malnutrition, road accidents and HIV can be prevented through carefully understanding the symptoms and context, and addressing the barriers to change.

It is important that locally specific assessment and analysis is carried out around the following factors before any prevention or response interventions are designed:

- a) Pre-existing cultural and social norms, roles and expectations on topics such as: day-to-day and household dynamics between men and women, boys and girls. Community perceptions on gender roles and towards sexual minorities, such as lesbian women, gay men, transgender and intersex individuals. Working on behaviour change communication at the community level for a prolonged time is a key preventative measure. For more information, please see the eCBHFA module on Behaviour Change Communication.
- b) Pre-existing survivor-centred referral services for health, psychosocial support, legal aid, and police services should be mapped, documented and made available. These sites should be visited in advance to ensure trained personnel are on site and a survivor-centred approach is used. Information about these services should be regularly updated. Having a widely available referral pathway is a preventative measure.

There are practical actions that individuals, families and communities can take in their own local places. Prevention and response require action from the full community.

Helping resources exist for children and adults who have been harmed by violence.

For use in the section on “Discussion”

Overview of interpersonal violence (“violence”)

Interpersonal violence is when one person uses his or her own power, in any setting, to cause harm physically, sexually or emotionally/psychologically or through neglect of another person or group of people. Interpersonal violence is a global humanitarian problem. It occurs in every continent, country and community. Violence against girls, boys, women and men happens in homes, schools, communities and online and the risk is increased in disasters. The impact of violence, and the trauma it causes, can be devastating for individuals, their families and communities and can last a whole lifetime or even generations. There can be physical, emotional and psychological impacts. Although it is complex, and often unheard and unseen, violence can be predictable and preventable.

Gender and violence

Gender inequality and discrimination are closely connected to violence, and can take the form of SGBV. Specifically, gender-based inequalities, gender-based discrimination and an abuse of power are root causes for violence. Girls and women are at particular risk of SGBV. A lack of access to education, based on gender, can reduce the ability to learn protection skills. Gender inequalities can lead to limitations including lack of trust, fear or lack of knowledge about where to get care in accessing health care, including psychosocial care. When violence does occur; and if all genders are not empowered to be safe from violence, the risk of violence increases.

Ecological model of violence – a comprehensive perspective

There is no single cause of violence. People hurt other people for many reasons and can be determined only through complex dynamics between individuals, their families, communities and societies. The factors that contribute to violence in any situation are called “social determinants” or “root causes”. These factors can include gender and age inequalities, cultural and social norms, a misuse or abuse of power, social exclusion, poverty, and alcohol and substance abuse. The ecological model shows how many factors at many levels combine to put people at risk. Although the determining of these factors can seem complex CBHFA volunteers can help to identify these determinants at the community level to prevent and respond to violence.

People involved in an act of violence

For each act of interpersonal violence there is a person inflicting violence, a target or victim/survivor of the violence, and often bystanders who watch, hear or know of the violence.

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The person inflicting violence must stop using violence and take responsibility for their violent actions. To help them get to this realisation, some persons inflicting violence may need to be made aware of help they can access and strategies they can apply when they are at risk of acting out violently. This is a process that requires reflection and a desire to work towards more peaceful solutions. The victim/survivor needs to get help for their physical injuries and their emotional injuries, but also strategies to help them prevent being caught up in another attack. Victims/survivors must know that the violence is not their fault and that help is available. The bystanders who witness the violence need to know they have an important role to prevent violence, speak out or report the violent act and get help once it is safe to do so. When bystanders intervene in a way that keeps them safe, the risk of violence happening or continuing is significantly decreased.

Convention on the Rights of the Child (CRC), Convention on the Elimination of Discrimination against Women (CEDAW), and Minimum Standards.

CRC, article 19, states that all children have the right to live lives free of violence; all of the CRC articles can be summarized into the following categories: protection, participation and provision. The SPHERE inter-agency Minimum Standards for Child Protection in Humanitarian Action reinforce these rights in emergencies.

CEDAW calls nation states to eliminate discrimination against women and girls. This includes violence. The Inter-Agency Standing Committee (IASC) standards for preventing and responding to SGBV outline how these rights can be achieved in emergencies.

United Nations Sustainable Development Goals (SDGs)

The SDGs include specific targets for addressing violence against girls, boys, women and men. These include:

- 5.2: Eliminate all forms of violence against women and girls;
- 5.3: Eliminate all harmful practices, such as child, early and forced marriage, and female genital mutilation;
- 16.1: Significantly reduce all forms of violence and related death rates everywhere; and
- 16.2: End abuse, exploitation, trafficking and all forms of violence against children

Local action (micro-projects)

The aim of this eCBHFA module is to help girls, boys, women and men, of all backgrounds, to take effective, sustainable local action, like through micro-projects, to prevent and respond to physical, emotional, gender-based and sexual violence, self-harm, and neglect. A micro-project is a small scale, focused, community-led project that can help to prevent or respond to violence.

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For instance, if a community is concerned about unknown people accessing a school when it is dark then putting up solar lights around the area might reduce the risk. Or, if a community believes that an understanding of local referral systems is not well known they might be supported to organize a community fair where agencies that provide helping resources share the work they do. (For more information, see micro-project guidelines).

Disasters and violence.

After a disaster, the risk of violence, especially SGBV and violence against children often increases. This may result from an accumulation of stress, loss and severe disruption to their daily lives. This includes: the collapse of protective systems; increased individual, family and community stress; changes in gender related power dynamics; a breakdown in services, such as health, legal, psycho-social support and security services; people relying on harmful coping mechanisms like alcohol, drugs and food for sex; and crowded and insecure environments, especially in temporary housing.

By understanding that a disaster and/or high-stress prolonged situation will likely result in violence, CBHFA volunteers can prepare their communities and themselves to prepare and put practical action in place as soon as disaster strikes to prevent and address violence in disasters.

Please refer to the *IFRC Minimum Standards on Protection, Gender and Inclusion in Emergencies* for more information and guidance.

The consequences/impact of violence

- Human: physical injuries; emotional/psychological injuries (lower self-esteem; shame, powerlessness, betrayal, guilt, etc.); loss of trust; disease and illness, etc.
- Economic: decrease to a country's gross domestic product (GDP) through loss of time from work; physical and mental health care costs; police, security, legal costs; loss of productivity, etc.
- Community development: fear to move in community; fear of access to community-services; lack of engagement in community or economy; reduced social cohesion, etc.

Psychosocial support and self-care for the helper

It is important to emphasize to participants that discussing the problem of violence or providing support to someone who has experienced violence, can be stressful and emotionally exhausting. Participants should ensure that they take care of themselves to limit secondary trauma, by taking time to discuss their feelings, take rest, and do things that they find healing or energizing.

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Survivor-centred approach

This module uses a survivor-centered approach: Everything begins with the experiences of the survivor. These experiences determine the need and the needs determine the services required. The safety of the survivor is prioritized and confidentiality is maintained at all times. The chart below details the key principles of a survivor-centred approach. ^{vii}

Safety	The safety of the survivor and survivor's family should be ensured at all times. Keeping survivors safe should be a number one priority. Survivors of violence are at heightened risk of on-going violence (e.g. domestic violence), murder or suicide, as well as social discrimination and isolation. Helpers have to assess safety risks and minimize the risks for survivors and their immediate family members.
Confidentiality	Confidentiality is paramount in all aspects of support for survivors. The threat of stigmatisation, social isolation and punishment is very real for those affected by violence. Maintaining confidentiality means that information about survivors should not be shared with others without the informed consent of the survivor. There are certain exceptions to this rule that are about the absolute safety of the survivor and/or immediate family (please see below). If information needs to be shared with another organization, always obtain the written consent of the survivor or of a parent or guardian if the survivor is a child. Informed consent means that the survivor will be informed about which information will be shared, with whom and for what reason. It is not ethical to share personal information about the survivor or their situation (e.g. giving their name or other identifying information) with anyone else – at home or in the workplace. Avoid identifying survivors of violence in the way services are provided. Survivors can be at risk of being identified by the community if they attend specialized programmes. This risk can be minimized by addressing the special needs of survivors of violence within broader protection, health and psychosocial programmes.

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Exceptions to maintaining confidentiality	▪ When there is a risk that survivors might try to hurt themselves ▪ When there is a risk that survivors might hurt others ▪ When a child is in danger ▪ When laws or policies require mandatory reporting (such as in the case of sexual exploitation and abuse by humanitarian staff).
Respect	The wishes, rights, and dignity of the survivor must always be respected. The survivor-centred approach empowers the survivor, with helpers offering assistance, facilitating recovery and providing resources for problem-solving but never taking any decisions for survivors. This can be especially difficult if a survivor decides to remain in the violent relationship or location. A lack of respect by helpers can increase survivors' feelings of helplessness and shame. It can prevent survivors from regaining a sense of control over their lives. It can reduce the overall effectiveness of interventions, and may even cause further harm.
Non-discrimination	All people have the right to the best possible assistance without unfair discrimination on the basis of gender, age, disability, race, colour, language, religious or political beliefs, sexual orientation, status or social class.

Survivor-centred referrals and local support systems

Support systems in a community can be formal or informal. Formal systems can include hospitals and clinics; the police; government ministries responsible for SGBV, child protection, psychosocial support; and professional counsellors, etc. Informal support can be drawn from traditional/local/village support systems; family; friends; hotlines; peer networks; women's groups; shelters for survivors of domestic violence; etc.

Local support systems and ways to access them (e.g. locations, hours, phone numbers, contact people, etc.) need to be identified in advance of delivering sessions with communities.

Trainer directions

DAY 1: Problem identification

Learning objectives

1. Become familiar with IFRC background on addressing violence and key concepts.
2. Understand the different types of violence.
3. Recognize the impact of violence.
4. Understand that violence can be predictable and preventable.
5. Know the tools and messages related to the eCBHFA module on violence prevention and response.

1. Introduction to Violence Prevention and Response (60 minutes)

1. Welcome participants to the session.
2. Encourage questions during the training. Explain that participants can also use paper to write questions and deposit them in the question box / stick them on the wall. The questions will be reviewed and addressed at the end of each topic.
3. Introduce the topic objectives and agenda. Open the session by asking participants:
 - Why do you think the community chose this topic?
 - Why is preventing and responding to violence so important?
 - Why is violence prevention and response included in eCBHFA?
4. Explain that:
 - the key objective of this three-day training is to prepare participants to deliver a two-day training to eCBHFA volunteers on the Violence Prevention and Response module (provide copy of the two day training content).
 - this training, and all of the trainings for this module, are expected to be interactive and build on participant knowledge. Most of the activities that participants will complete in this training will be the same that they will use to train eCBHFA volunteers.
 - before implementing this module, an assessment with the local community should be completed to understand the needs, challenges, and strengths for preventing and responding to violence; and to define local survivor-centred referral services to support survivors of violence.
 - after an assessment is completed, this module can be adjusted and used to raise awareness with communities and then the same communities can be supported to implement their own local micro-projects to address violence.

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5. Discuss the importance of all participants feeling safe during this training and when working with communities. Explain that violence impacts most people at some point in their life, and that talking about violence can trigger strong emotions and uncomfortable memories. Anyone in the training can take a break at any point. It is important everyone is supportive and creates a safe environment. Participants can be asked what makes a safe training environment for them.
6. Stress that the principle of **do no harm** is essential to this training and the delivery of the module in communities.
 - Ask the group to define "do no harm" (see "Topic Summary" for definition).
7. Complete the "treasure hunt" activity to introduce the participants to the messages of the lesson cards.
 - Break the group into smaller groups of 4 people each and ask each of them to answer the following questions.
 - Explain that the lesson cards are a tool to help reach communities with the key messages of the module. The lesson cards can be delivered in large community gatherings, house-to-house, in schools, or through other community forums. The messages can also be delivered without using the lesson cards when other approaches are more appropriate; approaches like theatre, art, conversations without the use of images, etc. The main thing is, the key messages are shared, people have time and space to discuss specific concerns, and communities are supported to find local solutions.

"Treasure hunt" questions	Lesson Card
a. What are the seven key messages of the Violence Prevention module?	1-7
b. What two cards are designed to take the longest time to deliver?	1-3
c. Which card discusses practical actions that can be taken to prevent sexual and gender-based violence?	4
d. Which card provides options for healthy discipline of children?	3
e. What card discusses, in detail, the role of gender in violence?	2
f. What card focuses on self-directed violence?	6
g. What cards talk about the risk of violence in disasters?	1-7
h. What card lists the impact of violence; list three examples?	1
i. What card outlines the different ways people can hurt each other emotionally?	5

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8. Stress that this module is best implemented as a coordinated effort with the community's leadership and with local and international partners as appropriate. By identifying and discussing the types of violence that are prevalent in the community, CBHFA staff and volunteers can identify the root causes that contribute to violence and tolerance for violence. As we learned in the behaviour change module, CBHFA volunteers can have a significant impact on addressing the individual, interpersonal and community levels – the areas of greatest violence prevalence globally. Working with others, like local government agencies, NGOs, and UN partners, we can have a higher impact.
9. Explain that violence is often a predictable and preventable problem. It is not inevitable and there is much that CBHFA volunteers can do to identify and address risks for violence.
10. Emphasize that the aim of this eCBHFA module is: To help girls, boys, women and men of all backgrounds, to take effective and sustainable local action to prevent and respond to violence such as SGBV, physical, psychological, neglect and self-harm.

2. Discussion (255 minutes)

Part 1 Overview of violence prevention and response (75 minutes)

For the discussions below, use the content in the "Topic Summary" section and documents from the "Materials" section to help answer questions and to provide additional detail.

1. Definition of violence. Working in pairs, have participants define "violence". Stress that a misuse of power is essential to violence.
2. Definition of gender. In pairs, have participants define "gender". Briefly discuss how "gender" and "sex" are different. Specifically, that gender is a cultural trait while sex is a biological trait.
3. Discuss the power of language, specifically, the differences between 'survivor' and 'victim' language. (Use the appropriate power point slide as needed to help debrief).
4. Briefly review: IFRC's strategic framework on violence prevention and response: Strategy 2020; Strategy on Violence Prevention, Mitigation and Response; Strategic Framework on Gender and Diversity; SGBV Prevention and Response Resolution; and various pledges and policies like the Code of Conduct and Child Protection Policy. Emphasize that addressing violence against children and SGBV are minimum standards in emergencies that need to be addressed or communities cannot truly be safe.
5. Discuss the importance of violence prevention and response within

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the United Nations Sustainable Development Goals and other UN frameworks like CEDAW and CRC.

6. Typology of violence. Explain the World Health Organization (WHO) typology of violence. Discuss how violence includes several categories: self-directed, interpersonal violence and collective violence (war).

Explain that each category of violence has common risk factors and are all connected together. Reinforce that the aim of this eCBHFA module is to focus mainly on interpersonal violence (people hurting each other), while it also includes messages on self-directed violence; the module is designed to help communities use a primary prevention approach (stopping the problem before it starts).

7. Review statistics on violence by asking participants to guess how many people are impacted by various forms of violence.
8. Discuss the continuum of violence. Explain that when children experience violence at an early age, the trauma can negatively impact all stages of their life.

“What happens when a child is 6 months or 6 years old can impact them when they are 60 or 86 years old?”.

9. eCBHFA volunteers learned about the ecological model in the Behaviour Change Core Module. Help the community explore the ecological model/comprehensive approach to preventing and responding to violence. Have participants conduct a visual, interactive example of the ecological model.

“Circles” activity

- a. Have one participant stand at the front of the room and represent the “individual” level. The participant can be given a fictional story to summarize their risk of violence, for instance: the person is a female child from a minority group, and she lives in an urban environment that is prone to disasters. Ask participants to brainstorm what makes an individual vulnerable to violence (e.g. gender, disability, marginalization, age...children are among the smallest and most dependent members of society, etc.).
- b. Brainstorm with the group what makes a person more vulnerable to be violent against others (e.g. a history of experiencing violence, anger management issues, alcohol and substance misuse, etc.).
- c. Have a few volunteer participants to come to the front of the room and surround the “individual” in a circle. The participants circling the individual represent “interpersonal relationships” (like family). Ask participants to brainstorm elements of violent relationships (e.g. poor parenting skills, family discord, gender inequality, alcohol and substance misuse, lack of resources and support). Remind participants that people who behave violently are not always violent but only in some situations and under certain conditions.

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- d. Ask several more participants to come to the front of the room and surround the “relationships” circle; so they stand outside as a ring circling them. This level represents the organizational. Explain that many of us belong to organizations like workplaces, religious groups, and sports and arts groups. Ask participants to consider what factors in an organization can increase the risk of violence (lack of protection systems, tolerance for violence, no/weak systems for reporting, etc.)
- e. Request several more participants to come to the front of the room and surround the “organizational” circle. This circle will represent “community.” Explain that people, families and organizations do not exist in a vacuum; they are part of communities. Ask participants to brainstorm the different factors in a community that contribute to violence (e.g. crime, gang activity, unemployment, lack of community services, acceptance of physical violence in schools, apathy, no prevention systems, etc.).
- f. Ask the remaining participants to come to the front of the room and surround the “community”. There should now be five circles of people around the “individual”. This last level is “policy.” Discuss how communities are part of a bigger society and culture that is influenced by policies. Ask participants to brainstorm how policies allow violence to happen (e.g. policies that accept economic inequality, gender and age inequality, social exclusion, or a lack of access to education and health systems, etc.).
- g. Explain that the dynamics between each level (e.g. circle) interact with each other and together increase the risk of violence.
- h. Discuss how at each level there is not only risk but also protective factors that help build resilience, or people’s ability to prevent or “bounce back” from violence. Emphasize how the opposite of each risk can be a protective factor (e.g. instead of social exclusion it becomes social inclusion; or, gender and age inequalities become equalities). Emphasize that the circles around the individual can work together to put the individual at risk or they can come together to protect the individual. This is in the hands of each of us and our families and communities.

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Part 2 Define specific types of violence (45 minutes)

For the discussions below, use the content in the “Topic Summary” section and documents from the “Materials” section to help answer questions and to provide additional detail.

Key questions.

- Divide the participants into 6-12 groups.
- Paste twelve pieces of flip-chart paper on the walls – on each piece of paper write one of the questions listed below.
- Ask each group to stand next to one piece of paper – and ask them to answer their question (on the paper) within five minutes.
- After the five minutes are complete, ask each group to rotate to a new piece of paper (going clock-wise) and take one-two minutes to add more answers to the last group’s answers.
- Continue until all groups have answered all, or most, questions.

Questions

1. What is emotional/psychological violence? Give examples.
2. What is sexual and gender-based violence? Give examples.
3. What is physical violence? Give examples.
4. What is neglect/deprivation? Give examples.
5. What is self-directed violence?
6. What role does gender have in violence?
7. What are alternatives to hitting children for discipline?
8. Which populations are the most vulnerable to violence? Why?
9. What role do “bystanders” have in preventing violence?
10. Why does the risk of violence increase in disasters?
11. What gives some people more power than others? How can power be misused?

- Summarize. Have each group return to their “starting flip chart” and pick out two critical points to share with the larger group. As a group discuss any issues that were challenging, surprises, or are of particular importance. (Refer to the lesson cards where necessary to show how the answers are reflected in the actual eCBHFA tools).
- Highlight that each of these forms of violence can occur not only in the community but also against Red Cross Red Crescent volunteers; in their work with the Red Cross Red Crescent from communities while they deliver programs, or in their own families. Recognizing that volunteers who deliver the eCBHFA Violence Prevention and Response module can be vulnerable is important and should be considered in the delivery of the module.

Part 3 Explore, in detail, each type of violence (135 minutes)

For the discussions below, use the content in the “Topic Summary” section and documents from the “Materials” section to help answer questions and to provide additional detail.

1. Detailed discussion. Explain that the group will now spend time discussing different forms of violence in more detail.

2. Emotional/psychological violence.

- Review the definition of psychological violence and give examples (from the flip-charts).
- Describe how emotional violence accompanies all other forms of violence (physical, sexual, and neglect), yet it can also stand alone. Stress that the impact of psychological/emotional violence can last longer, and take more time to heal, than physical injuries.

3. SGBV

- Show an introductory video like “Lets change the story”² from Australia, or a local video from your own location. The short IFRC introduction to SGBV can also be shown:
<https://www.youtube.com/watch?v=O5KnUVj0jwc>.
- As a large group, define SGBV (show power point) and explain its various forms (using hand-out on definitions).
- Conduct activity on facts and myths about gender and abuse of power (power point).
- Discuss the factors that increase SGBV. Note that SGBV is often violence against women and girls (because of cultural values that treat women and girls as less important than boys and men); however, SGBV can also be against boys and men.
- Break the participants into four groups and conduct the activity on definitions of SGBV: “How do I define the types of SGBV?” (power point).
- Most SGBV, against all genders and ages of people, is most frequently committed by men. Men need to take responsibility for their behaviour. This module may help people to recognise these issues but volunteers should not take it upon themselves to ‘fix’ or heal violent men. However, they can help them be aware of their actions and how it affects them those they love and their environment.
- Ask: “What are the consequences when women and girls are not treated equally to men and boys?”
- Consider, what role do men and boys have in ensuring equality and safety for all genders? How can we encourage these changes among men and boys?

² Our Watch, the Victorian Health Promotion Foundation (VicHealth) and Australia’s National Research Organisation for Women’s Safety (ANROWS).

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- Discuss the role of secrecy in SGBV.
- Discuss the difference between consent and cooperation.
For example, for sexual violence the issue of consent is the determining legal factor. In order for consent there must be equal knowledge, power and support systems. Emphasize that children are unable to give consent because they do not have equal power, knowledge or support systems. A boss pushing his employee for sexual activity or an aid worker pressing a community member for sex are also examples of imbalances of power where sexual abuse can happen. Consent gained from pestering or threatening the individual with less power is sexual abuse or violence. Cooperation by someone (child or adult) to do a sexual act or something they do not want to do may be a survival tactic, especially if they are or feel threatened, in immediate danger, or dependent on the person doing the harm.
- Refer to the Mental Health and Psychosocial Support module for more guidance on working with distressed populations struggling emotionally and of course, refer people in danger for support and help.

Reporting concerns

Any concerns about violence or harm against a child or adult by a Red Cross Red Crescent volunteer, staff, contractor, consultant or partner needs to be reported. If it is not the safety of the child can continue to be put at risk and a child needing help may not receive the protection they need. Concerns can be reported to your line manager; or a senior manager; or Human Resources; or Safe Call, a global hotline available for IFRC personnel: +442076965652, ifrc@safecall.co.uk, www.safecall.co.uk/confidential-contact.php.

Any concerns relating to the protection of a child in the community needs to be reported. Even if local laws do not require that concerns about violence against children be reported, Red Cross and Red Crescent personnel are obligated to report and follow local laws. Mapping, documenting and making available local SGBV referral pathways is an important step in achieving this. See your National Society Child Protection Policy or government policy for details.

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4. Physical violence.

- Review the definition and examples of physical violence (from the flip-charts).
- Explain that people of all ages, genders and backgrounds can experience physical violence.
- While anyone can be hurt by physical violence, children are at higher risk because often people confuse discipline with violence or the discipline turns into violence. Many of the reasons used to justify hitting children represent justifications for also hurting adults. Therefore, physical violence against children is explored in detail in this section.
- Emphasize that violence against children most often happens in homes, schools and institutions when adults confuse physical force with discipline. Highlight the differences between violence and discipline: discipline is about teaching...violence is about fear and hurt. Explain that in the same way that adults cannot learn when they are scared, hurt or angry, children also cannot learn well when they have been hit because they are scared, hurt and angry.
- Break the participants into small groups. Ask each group to take ten minutes to develop a response for each of the following myths.

Myth 1: Hitting is the best option.

Myth 2: Hitting is needed as a "last resort".

Myth 3: Hitting is harmless.

Myth 4: Hitting some parts of the body is okay.

Myth 5: Most adults only hit in serious situations.

- Go through each myth and ask participants to present their answers. Below are responses to the myths that can help guide the discussion.

Myth 1: Hitting is the best option

FALSE. Research shows that hitting children is not effective in changing children's behaviour in the long-term. Methods for teaching children discipline, respect, and good decision-making include setting clear standards for what is expected, providing lots of love and affection, explaining things to the child, and recognizing and rewarding good behaviour.

Although hitting children might have short term results, it does not change long-term behaviour; it hurts, scares and humiliates children. Hitting children is ineffective and unnecessary and often causes fear and distrust from the child, leading to further discipline issues as the child feels more angry and frustrated. As a parent or authority figure, you are serving as a model of behaviour. If you hit, you are modelling that hitting is an acceptable response. Be a good role model.

Myth 2: Hitting is needed as a “last resort”.

FALSE. There are no situations where hitting children is the only option. In fact, when children are hit as a “last resort” it may be even more harmful because by then adults may be very upset and act with little control. When children are hit as a “last resort” they learn that hitting is okay when people are angry.

Myth 3: Hitting is harmless.

FALSE. When someone says, “I was hit as a child, and I am okay”, they are arguing that hitting does no harm, or is even good for children. This is not supported by research.

Often the harmful effects of hitting are not recognized because: many adults find it hard to admit that their parents hurt them; it is difficult to acknowledge that what they are doing to their children has long term negative consequences; and the impact of hitting children may not be evident immediately.

Hitting children once, twice or often can have harmful effects on the child. Some of the effects can be immediate, like bruising, fear, anger, shame, humiliation; other impacts can be long-term like lack of trust, depression, poor health, anger and using violence to solve problems.

Myth 4: Hitting some parts of the body is okay.

FALSE. No part of a child’s body should be hit. Hitting hurts, scares, creates trauma, and humiliates the child no matter what part of the body is hit. Hitting does not change behaviour in the long term.

Myth 5: Most adults only hit in serious situations.

FALSE. A lot of adults use violence out of their own frustration; they use their power to hit children for any misbehaviour. They hit when their patience runs out and it seems like the simplest solution; they don’t stop to think of how to best teach this lesson without harming the child.

Tip: If participants still feel it is okay to hit a child, ask all those who like to be hit to stand; then ask all those who think it is okay to hit an adult to stand. When no one stands, ask “Why is it any different for a child?”

- Summarize: Explain that there are many healthy and safe options to discipline children. Building on a strong bond of affection between parents and children, adults can choose effective and respectful discipline methods. Refer participants to lesson cards on physical violence and emotional violence and for a list of options for healthy discipline. Participants can add other ideas.

Summary of options for healthy discipline

- Discipline when you are calm – do not discipline if you are angry.
- Separate yourself from a child when you are very angry; return to talk to the child when you are calm.
- Use positive language – using hurtful words and insults is not effective or necessary.
- Criticize the actions of the child – not the child him/herself.
Instead of saying, 'You are very bad' which attacks who the child is, say 'You have behaved very badly.' Or 'I am very disappointed in your actions.' When you criticize the child, you leave little room for them to improve, whereas if you criticize their actions, you are not condemning them personally but indicating how they can improve their behaviour to get your approval.
- Use reasonable consequences that are appropriate to the misbehaviour
- Model the behaviour you want your children, family and friends to follow
- Remember discipline means "to teach"; use each situation as a teaching opportunity.

5. Neglect

- Highlight that neglect of basic needs can happen to anyone, yet children, the elderly and people with disabilities can be impacted the most. The risk of neglect is escalated when families are under stress and struggling to cope.
- Ask participants to list five basic rights all children have according to the Convention on the Rights of the Child.

Examples of children's basic rights

Safety and protection, nutrition, shelter, good hygiene, medical and dental care, adequate education, moral guidance and discipline.

- Brainstorm as a group how the needs of the elderly may also be neglected.
- As a large group, discuss specific activities that can be taken to prevent neglect.

6. Self-directed violence

- Review the definition of self-directed violence.
- Explain that self-directed violence is a common problem across the world that can be prevented.
- Break participants into small groups to answer the following questions: a) what could motivate someone to hurt their own self? b) how could someone hurt their own self? c) what actions can be taken to prevent the risk of self-harm? d) what might become barriers to prevention? e) when someone self-harms, others may copy the self-harming behaviour. How can we best respond after the initial self-harming incident to prevent 'copycat self-harmers'?

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- As a group discuss:
 - What can motivate: Self-directed violence happens when someone believes, for various reasons, that hurting their own body is the only solution to their sadness, illness or loneliness. This can be done through “cutting” and other self mutilation, ingesting harmful substances, or attempting to take their own lives. The person is losing, or has lost, hope.
 - How can someone hurt their own self? Use examples from the flip-charts (from the earlier activity).
 - What actions can be taken to prevent risk: See lesson card #6 for specific actions.
 - Discuss how there can be cultural, religious or other barriers that make talking about or responding to self-directed violence very difficult. Overcoming stigma is essential to prevent and respond to self-directed violence.
- Emphasize that individuals can show signs, or try several times, before hurting themselves or ending their own lives.
- Explain that the family and friends of a person who has committed suicide will require social and professional support. Self-harm is a complex form of violence that requires PFA and beyond.
- Remind participants, that self-directed violence is a complex problem, and often our best role in the Red Cross Red Crescent is to listen and help someone at risk access qualified local support systems.

7. Violence in disasters

- Review the responses (on the flip chart) about why violence can increase in disasters.
- Ask “what practical actions can be taken to reduce/mitigate violence in disasters?” What makes addressing violence in disasters so complicated?

Sample actions to mitigate violence in disasters:

Do not act out violently in anger or fear.

Manage your stress levels through talking to others, meditation or deep breathing, keeping your hands and mind active, helping others, taking time for yourself whenever possible.

Do not use harmful coping strategies like alcohol or drugs.

Find people who can support you emotionally, spiritually, and physically.

Make a plan so you and your family know how and where to go to be safe; plan how your family can communicate and re-connect after a disaster.

Work with community leaders, organizations and schools to build violence prevention into disaster planning and across all disaster response sectors (e.g. Health, WASH, Livelihoods, etc.).

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8. Vulnerable populations

- Summarize that all people can be vulnerable to violence, although some populations can be at particular risk based on local circumstances. Among the most vulnerable often include children, the elderly, people with disabilities, migrants, and anyone who is considered “different” or “less important” than the rest of the population.

9. The consequences/impact of violence

- Ask participants to brainstorm the physical and emotional impact of violence. Explain that the impact can have human, economic, and community development dimensions.
- Emphasize that human impacts include stress on relationships, trauma, psychosocial and health repercussions, and a questioning of personal self-worth.
- Remind one another that the impact of violence can be deep and profound, even if it is not visible to others. There is no formula to measure human suffering, yet the pain can be felt across a lifetime and even impact the lives of future generations.

Facilitator tip:

In discussions, remember:

- Violence is often kept secret and hidden; it is not always seen by others. It can be *unseen* and *unheard*.
- The impact of violence can be different for each individual based on many factors. No one form of violence necessarily has a greater impact than others.
- People who inflict violence need to take responsibility for their own behaviour; this includes when men are violent towards other men, women or children.
- The Red Cross Red Crescent has an important role to address violence through our work in communities and through our influence with key decision makers at all levels.
- By working with communities, we can help identify practical solutions to prevent violence that are appropriate to the local realities. We can all take action within ourselves, families and communities.
- Violence is a *predictable and preventable problem*. This is supported by growing evidence from around the world.

3. Closing (15 minutes)

1. Summarize the key points of the day.
2. Gather feedback from participants about the day.

DAY 2: Practicing implementation

Learning objectives

1. Understand the content of the Lesson Cards.
2. Practice and become comfortable using the Lesson Cards.

4. Check your understanding (15 minutes)

1. Ask participants to review the main learning points of the training from day 1.
2. Ask participants to work in pairs to complete the self-assessment; the self-assessment is to check their understanding of the information covered so far.
3. Allow enough time for everyone to complete the self-assessment.
4. Have participants check the answers against the answers below.

Answer key:

1. What are alternatives to hitting children when they need discipline:
 - Discipline when you are calm – do not discipline when you are angry
 - Separate yourself from people you are upset with when you are very angry; return to talk to them when you are calm
 - Use positive language – using hurtful words and insults is not effective or necessary
 - Model the behaviour you want your children, family and friends to follow
 - Remember discipline means “to teach”; use each situation as a teaching opportunity.
2. What are some situations in a family when it is okay to hit someone else?
 - It is never okay or necessary to hit someone. Violence is not safe, effective or necessary. Violence should not be used in a family or in any situation.
3. What are common ways that gender relates to violence?
 - Gender-based inequalities and gender-based discrimination can be root causes for violence
 - A lack of access to education, based on gender, can reduce the ability to learn protection skills
 - Gender inequalities can lead to limitations in accessing health care, including psychosocial care, when violence does occur
 - If all genders are not empowered to be safe from violence, their risk of violence increases.
4. What are some of the human impacts of violence?
 - Physical injuries
 - Emotional injuries like feelings of shame, stigma, exclusion, trauma, powerlessness and betrayal, confusion, sadness, and anger
 - Loss of trust and self-esteem
 - Diseases/illness.

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5. If you watch or hear someone being sexually hurt, what immediate action can you take?
 - Get the person being hurt to safety
 - Get help immediately
 - Speak up to bring attention to the violence
 - Make it clear to the inflictor that violence is unacceptable and must stop immediately
 - Talk to someone else in the home or community who can help.
6. What practical actions can you take to prevent violence in a disaster?
 - Do not act out violently in anger or fear
 - Manage your stress levels through talking to others, meditation or deep breathing, keeping your hands and mind active, helping others, taking time for yourself whenever possible
 - Do not use harmful coping strategies like alcohol or drugs
 - Find people who can support you emotionally and physically
 - Make a plan so you and your family know how and where to go to be safe; plan how your family can communicate and re-connect after a disaster
 - Work with community leaders, organizations and schools to build violence prevention into disaster planning.

5. Lesson learned (60 minutes)

1. Provide an overview of key lessons learned by other National Societies in implementing the eCBHFA Violence Prevention and Response module. Focus on innovative, replicable, and scalable approaches.
2. A sample lesson: The module can be framed in any way that the community chooses and is locally acceptable. If language for "violence prevention and response" is a barrier, then the module can be framed as a project for "community safety" or "friendly environments" or "protection of the vulnerable".
3. Where possible use videos to highlight lessons. For example: this video from the Pakistan Red Crescent: <https://youtu.be/RVzjSuW42ys>.

6. Ways of learning (30 minutes)

1. Explain that different people can have different ways of learning. It is important for facilitators to communicate their messages, especially for a sensitive topic like violence, in a variety of ways so that different ways of learning can be engaged.
2. Ways of learning include: linguistic (e.g. talking), logical (e.g. mathematical), musical (e.g. song or sound), visual/spatial (e.g. images), physical (e.g. body movement), interpersonal (e.g. with others), intrapersonal (e.g. self-reflection), and naturalistic (e.g. in nature).
3. Ask participants to practice communicating the 7 key messages from the lesson cards using at least three different ways of learning.

7. Activities to practice (210 minutes)

Review the key learning points: have participants state one thing they learned from the first day of training.

Key learning points

1. Everyone deserves to be safe from all violence: physical, sexual, psychological, and neglect.
2. Girls, boys, women and men need to be treated equally and with respect.
3. Hitting another person is harmful and abuses their human rights.
4. Sexual and gender-based violence should never be tolerated, it is a violation of our basic human rights: dignity and safety.
5. Psychological violence is damaging to a person's self-esteem, it often has long-lasting consequences.
6. Harming yourself hurts you and those you care about.
7. Violence can be prevented – help and laws exist.

1. Review/practice the community activity using the lesson cards.
 - Explain that the aim of this module is to support communities to take local, practical, sustainable action to prevent and respond to violence. The lesson cards help provide standardized messages, and introduce key concepts. The lesson cards are designed to be delivered at the household or community level. However, they can also be used in creative ways through community activities like theatre, song and dance, etc.
 - After the lesson cards or key messages are discussed with a community and they are supported to identify local action, the Red Cross Red Crescent can determine what role it has to help communities implement their local activities (see Guidelines for implementing violence prevention and response micro-projects).
 - It can be useful to consider implementing lesson card 4 (sexual violence) in separate groups of females and males. This may allow more open discussion and create a safer environment for the community participants.

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Activity:

Split the participants into four groups and ask each group to practice delivery of one of the Violence Prevention and Response lesson cards 1-6 (except lesson card 7).

- a. Assign each group one lesson card (except lesson card 7) which they will be responsible to present.
- b. Provide each group with 30 minutes to practice delivering their lesson card. Ask the groups to ensure their presentations (using the directions and content on the back of the lesson cards) are interactive using multiple “ways of knowing”, involve all members of the group, and engage the audience. Each group should suggest one locally-led micro-project to reinforce their lesson card and explain why the micro-project is important (see Guidelines for implementing violence prevention and response micro-projects).
- c. Ask each group to deliver in twenty minutes the key elements of their lesson card to the whole group. After each presentation, have each member of the “audience” fill out one post-it note what they liked about the presentation and one post-it note on what changes they would recommend; have participants post the notes on flip chart set up for each group (lesson card).
- d. After all groups have finished, have each group review the post-it notes on their flip chart and discuss as a group their learning based on the feedback.
- e. Debrief themes about what worked, what did not, and answer any other questions the participants may have.
- f. Highlight that the learning is best when it involves questions, participation and clarity and it is practical and appropriate to the local context.

Facilitator tip:

Common issues to consider in preparing to deliver the lesson cards:

- Avoid lecture-style presentations with communities; the lecture approach, where one person stands in front of a group and talks without interaction will not lead to the participants internalizing the information.
- The lesson card on SGBV, in particular, can create discomfort among volunteers and community participants. It can help to discuss with community leaders how to deliver sensitive messages before sessions with communities are organized.
- Violence can be a sensitive topic; it is essential to know that some participants may have experienced violence in their own lives. If someone discloses violence, listen carefully to the person and refer them to local support systems.
- Engaging men in the sessions can be a challenge. They may be working, attending religious or social events, or they may not perceive the session to be relevant or interesting. To reach men it might mean being creative by working with men who can be models and peer educators for others, working with religious leaders, and using sports and cultural forums.

8. Closing (15 minutes)

1. Summarize the key points of the day.
2. Gather feedback from participants about the day.

DAY 3: Planning action

Learning objectives

1. Become familiar with local laws and community support services.
2. Recognize how to handle disclosures of violence and make appropriate referrals.
3. Identify actions that can be taken locally to implement the eCBHFA module.

9. Check your understanding (15 minutes)

1. Review key points from day 2, including key lessons from the practice delivery of the lesson cards.
2. Discuss any key questions that remain unanswered from day 2.

10. Protection laws (30 minutes)

For the discussions below, use the content in the “Topic Summary” section and documents from the “Materials” section to help answer questions and to provide additional detail.

1. Have participants look at Lesson Card 7 (Violence is preventable; helping resources and laws exist.).
2. Briefly review key conventions, laws and minimum standards: Convention on Rights of the Child, article 19; CEDAW; national legislation; provincial/state/territorial legislation; and global minimum standards for Child Protection in Humanitarian Action and Preventing and Responding to SGBV in emergencies.
3. Ask participants to identify when their country signed the Convention on the Rights of the Child and CEDAW.
4. Have participants identify the local laws in their country to address violence against girls, boys, women and men.

11. Community support systems (30 minutes)

For the discussion on community support systems, use the content in the “Topic Summary” section and documents from the “Materials” section to help answer questions and to provide additional detail.

1. Have participants work in pairs to define formal and non-formal community support systems in their communities.
2. How can community services be accessed? In the community are there any existing referral cards or other tools to hand out to communities so they know where to access help if violence happens?
3. Stress the importance, as part of the “do no harm” principle of identifying community support systems before sessions with communities start.

12. Handling disclosures (120 minutes)

Remember: Before any training, local referrals for SGBV and violence against children need to be mapped and available for participants. When volunteers deliver messages and programming in the community they also need to first have in available information on local referral services.

For the discussions below, use the content in the “Topic Summary” section and documents from the “Materials” section to help answer questions and to provide additional detail.

1. Explain that helping someone hurt by violence is very important.
2. Remind participants that sometimes when we talk about violence, people may recognize that they themselves have experienced violence in their own lives. Therefore, it is important everyone feels safe.
3. Emphasize that it is our responsibility to be able to provide information on where to get support, and to act in a responsible and supportive way: listen and refer. Remember, it takes courage to share an experience of violence!
4. Explain that it is not the role of the Red Cross Red Crescent is not to investigate but to help make sure that the victim/survivor of violence is safe; our role is to take appropriate ACTION as outlined in the steps below. The main role is to listen and refer.

Handling disclosures of violence, ACTION steps:

Action 1: Acknowledge the person’s situation and feelings. Listen carefully to what they say.

Action 2: Comfort the person and ensure they are safe.

Action 3: Document what you hear and what you see. If it is a child disclosing, you need to report immediately to the appropriate agencies. If it is an adult, it is their choice if they will seek help or not.

In essence: **Listen and refer.**

5. Emphasize that the difference between adults and children is that children must be supported to get help according to local child protection laws. For adults, it is their own choice whether they want support or not.
6. Highlight that handling a disclosure should take a survivor focused approach. It is important to begin with the experiences of the

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survivor. These experiences determine the need and the needs determine the services required. There four key principles to follow for a survivor-focused approach:

- Safety; confidentiality; respect; and non-discrimination. These need to be maintained at all times.
- Discuss each of the principles and how they apply during a disclosure of violence.
- Explore what the terms “safety” and “confidentiality” mean in the local context. How can safety and confidentiality be prioritized in sessions with communities?

7. Provide participants with several scenarios to practice how they would respond.
- Define where disclosures may happen: during education session, after sessions, in the community.
 - Break participants into groups of four.
 - Have each group discuss the scenarios and brainstorm practical ways that they could respond to the scenarios while applying the four principles of a survivor-based approach.

Scenarios:

After an eCBHFA session, a participant tells you her husband sometimes physically attacks her.

- (Listen, acknowledge the person’s courage for sharing, and provide referrals services to local helping resources. Afterward, take time for yourself and do something that will help you process the experience).

A community leader tells you she has heard rumours that children are being sexually abused by a specific person in the community.

- (Listen, acknowledge the person’s courage for sharing, and provide referrals services to local helping resources. Afterward, take time for yourself and do something that will help you process the experience).

In the middle of an eCBHFA session, a participant says he has recently considered harming himself and that he has not told anyone before this.

- (Listen, acknowledge the person’s courage for sharing, ask that you discuss more at the next break (in a safe place away from the rest of the group). Do not encourage the group discussion to become about personal experiences. At the break, listen and refer the person to local helping resources. Afterward, take time for yourself and do something that will help you process the experience).

- In the larger group. Discuss the actions that groups identified.
- Reinforce: Do LISTEN and Do be compassionate; but No secrets. No surprises. No promises. No interviews. No assumptions. In essence: listen and refer.

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8. Explain that, in addition to referrals, helpers can sometimes provide practical help to survivors of violence. When someone has experienced a crisis situation or is in shock, it is a great help if someone offers practical support. A few examples include:^{ix}
 - contacting someone who can be with the person
 - arranging for children to be picked up from school
 - making sure the person is warm and provided with food and drinks if there is a need
 - helping the person with transport home
 - helping them to get to the hospital or other support as needed.

Remember, at the same time, to respect the person's wishes and not to take over too much responsibility. Support them to regain control of their own situation, to consider their options and take their own decisions.

9. Have each small group review Lesson Card 7 and discuss how they would teach it.

13. Expert resources (60 minutes)

1. Invite a local helping resource person or partner agency to present on their services for victims/survivors of violence.
 - Helping resources may include: representatives from the police, ministries of women and children's services, health and psychosocial providers, religious leaders, and local NGOs, etc.
 - Before the panel starts, stress that this module is best implemented as a coordinated effort with other local and international partners. On our own we cannot prevent and respond to all violence, but working with others like local government agencies, NGOs, and UN partners, we can have a higher impact and make the most of each agency's strengths.
 - Provide time for one or more presentations from helping resource providers. Then allow at least 30 minutes for a question and answer period.

14. Planning for local action (90 minutes)

1. As a large group, brainstorm what risks might arise from delivering this module (for example, safety of volunteers, increased risk of violence in homes if women feel empowered but men resent this, support systems do not respond well, etc.). Discuss solutions to mitigate these risks (e.g. working with community leaders from the start, having referral systems in place and ensuring they are effective, etc.).
2. Barriers.

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Discuss barriers to preventing violence in families and communities. Have participants put one barrier on a post-it note and post on flip chart paper; e.g. lack of understanding, weak protection systems, few resources, long histories of violence in the family or community, et

"Facing chairs" activity

Place two long rows of chairs facing each other, so each facilitator is facing a partner.

State one barrier from the post-it notes and ask participants to discuss with their partner (person sitting in the row across from them) how they could overcome this barrier.

Have each facilitator switch partners (one row moves one chair to the right). And have the pairs discuss a solution to a new barrier. Switch groups one or two more times.

Debrief the barriers as a group and highlight key themes.

3. Define next steps.

- When will an assessment be conducted? Who will do it and where? How can communities be engaged in that process? How will the information be analysed?
- Who can lead the translation of the materials?
- Who can design artwork to reflect the appropriate local customs and realities? How long will this take? Who in the community and among local partners can provide feedback?
- Are local referral systems already identified or does that need to still happen? (Referral systems need to be in place before sessions with communities start)
- When will participants deliver the Two Day trainings?
- Who specifically will they train? How will they engage men in the process?
- Where will they train?
- How will micro-projects be included in the process?
- What steps can be taken to achieve the principle of "do no harm"? Including avoiding discrimination against any group (e.g. by leaving them out), vulnerable groups are not put at higher risk of violence, and ensuring volunteers are safe from violence when they deliver the module?
- What agencies can be local partners?
- What is their specific, measurable, achievable and time-bound plan for the next one month? Next six months?
- Ask participants to present their plans.

15. Closing (30 minutes)

1. Personal actions.
 - Ask what personal actions participants can take to prevent violence personally, in their family or in their community.
 - Have participants write out on a post-it note a personal pledge of action they can take to promote nonviolence.
 - Participants can share with others their pledges, if they choose.
2. Answer any final questions.
3. Summarize the Main Learning Points.

Key learning points

1. Everyone deserves to be safe from all violence: physical, sexual, psychological, and neglect.
 2. Girls, boys, women and men need to be treated equally and with respect.
 3. Hitting another person is harmful and abuses their human rights.
 4. Sexual and gender-based violence should never be tolerated, it is a violation of our basic human rights: dignity and safety.
 5. Psychological violence is damaging to a person's self-esteem, it often has long-lasting consequences.
 6. Harming yourself hurts you and those you care about.
 7. Violence can be prevented – help and laws exist.
-
4. Remind the group that our communities cannot prosper if people are scared or hurting in their mind, body or spirit. The ability to prevent and respond to violence is in our hands, there is nothing inevitable about people suffering from violence. All of us can take meaningful action and inspire others to also find solutions.
 5. Have participants fill out the Satisfaction Questionnaire.
 6. Thank everyone for their participation in the training.

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